

RADIOLOGY UNIT

Dr. B. R. Ambedkar Institute Rotary Cancer Hospital
All India Institute of Medical Sciences, New Delhi-110029

[REDACTED]	Age/Gender: 56 Years and 3 Months / F
[REDACTED]	[REDACTED]
Accession No : 580793	Location : MO OPD
Date of Examination : 20-MAR-2018 10:12 AM	

Procedure: CECT of Chest, Abdomen & Pelvis.

Clinical background: Metastatic breast cancer

Chest

Serosal spaces: Multiple nodular pleural deposits are seen involving costal/mediastinal, diaphragmatic surfaces and fissural right pleura with loculated right pleural effusion. No pericardial effusion.

Deposit along the mediastinal pleura shows loss of fat plane with pericardium at places. Both lungs: Two tiny subpleural nodules are seen in lingula and superior segment of LLL. There is small volume of right hemithorax with pleural nodules encasing the right lung causing scalloping of lung outline and architectural distortion.

Mediastinum: Few enlarged prevascular lymph nodes (12mm SAD) and in retrosternal location (15mm SAD) Trachea, main bronchi: Normal.

Lytic sclerotic changes with associated soft tissue component are seen involving sternum with multiple sclerotic lesions in vertebrae, pelvis and femur

Abdomen & Pelvis

Liver:/CBD/ Gall bladder/Pancreas/Spleen: Normal. Both kidneys/ ureters: Normal, No hydronephrosis.

Retroperitoneum/ vessels: Enlarged necrotic retrocaval lymph node is seen with SAD 16mm.

Free fluid: No ascites.

Urinary bladder/Uterus/ adnexa: Normal.

Additional information: Bilateral bulky adrenals. Suspicious small lesion in outer quadrant of left breast (7mm)

Comparison: as compared to previous scan dated 20-11-2017, there is no significant change in the size and extent of the deposits.

Impression: Post op status of right breast with pleural, retroperitoneal, mediastinal nodes, adrenal and bone metastases as described. Stable disease

Radiologist: Dr. Ashwin Desmukh (SR) / Dr. Mukesh Kumar

Dr. B.R. Ambedkar Institute Rotary Cancer Hospital
ALL INDIA INSTITUTE OF MEDICAL SCIENCES

Height.
Weight
BSA

Name of the patient

Flowsheet Medical Oncology

PATIENT DATE

Date :

Temp Heart Rate Resp Date B.P. Resp. Sys. CVS CNS						
TREATMENT Drugs 1. 2. 3. 4. 5. 6. 7.						
LABORATORY HB/PCV Platelates (K 1000) WBC (X 1000) ANC ESR Blood Sugar Blood Urea/Creatinine Calcium/Phosphate Uric Acid (mg %) Sodium / Potassium Billirubin (mg %) D/I Total Protein (%) A/G Ratio SGOT/SGPT ALK. Phos (Units) Tumour Markers Others	21/2/18 11.0 5.09L 10830 P34 L18 E1M7 ANC=7992	24/3 9.4 4.32L 10460 N7047	8/4/18 9.9 610X10 ³ 114W N-76	10/4/18 10.1 532X10 ³ 9W0 N-87	25/18. 8.6 7100. N842	
REACTIONS TO THEARAPY Nausea Diarrhea Mucositis Fever Others						
SUPPORT DATA BT / PRP I/O						

18F-FDG WHOLE BODY PET-CT STUDY

Age/Sex: 55Y/F

Study ID: FDGN/14597/17

Date: 28.12.2017

Indication: Metastatic carcinoma right breast, post surgery (2004), recurrence (skeletal, lung, pancreas, lymph nodal and adrenal metastases), post chemotherapy.

Procedure: PET-CT acquisition was done 60 minutes after injection of 10 mCi ^{18}F -FDG by intravenous route, from the level of orbits to mid-thigh. CT was done for attenuation correction and anatomical localization.

PET-CT Findings:

Head and Neck: Increased FDG uptake noted in bilateral palatine tonsils with few subcentimetric mildly FDG avid bilateral level II lymph nodes – likely infective. Visualized paranasal sinuses, skull base, pharynx, larynx and thyroid do not show any abnormality on CT.

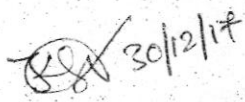
Thorax: Increased FDG uptake is seen in soft tissue thickening (1.0cm) over sternum at the level of right 3rd rib and soft tissue density deposit inferior to the lower end of sternum. Also seen irregular thickening in the left breast - ?significance. FDG avid sub-centimetric left axillary lymph nodes are seen. Diffuse irregular right pleural thickening with increased FDG uptake and right pleural effusion is seen. FDG avid mass like consolidation of size 4.6x2.3cm is seen in right lung middle lobe. FDG avid prevascular, paratracheal, subcarinal, left peribronchial and hilar lymph nodes of variable sizes are seen, largest prevascular lymph node, 2.3x1.6cm. Physiologic FDG uptake is seen in the myocardium.

Abdomen-Pelvis: Bilateral adrenal glands are bulky and showing increased FDG uptake. Few FDG avid peripancreatic, aortocaval (2.2x1.1cm) and retrocaval lymph nodes are seen. Normal FDG distribution is noted in the liver, spleen, gastrointestinal tract, kidneys and urinary bladder. Liver, biliary ducts, gall bladder, spleen, kidneys, stomach, bowel and urinary bladder appear normal on CT. No ascites is noted.

Musculo-Skeletal System: Lytic sclerotic lesions (predominantly sclerotic) with increased FDG uptake are seen in left clavicle, sternum, right 2nd, left 5th and 9th, C3, D7, 12, L2,4 vertebrae, bilateral pelvic bones, sacrum and bilateral femora.

Impression:

- Metabolically active disease presternal region – local recurrence with metastases to mediastinal, retroperitoneal lymph nodes, bilateral adrenal glands, multiple skeletal sites with right pleural effusion and pleural thickening.
- Also noted mild irregular thickening in the left breast parenchyma with metabolically active left axillary lymph nodes – needs further evaluation.
- CD not provided for comparison – treatment response evaluation cannot be made.


Dr K S Reddy
Senior Resident


Prof Rakesh Kumar
Consultant



प्रयोगशाला अर्बुद विज्ञान, डॉ भीमराव अम्बेडकर संस्थान रोटरी कैंसर अस्पताल अखिल
भारतीय आयुर्विज्ञान संस्थान नयी दिल्ली -110029
LABORATORY ONCOLOGY, Dr B.R.A. Institute Rotary Cancer Hospital All India
Institute of Medical Sciences, New Delhi-110029

UHID: [REDACTED] Reg Date : 08/12/2016 08:39 AM
Patient Name : [REDACTED]
Sex : Female Age : 55 years 9 months 10 days
Department : Medical Oncology Unit Name : 192.168.15.8/ehospital/laboratory/lab_report_print.jsp
Unit Incharge : Dr. Lalit Kumar (HOD) Sample Collection Date: 18/09/2017 03:47 PM
Lab Name: Oncology Lab Lab Sub Centre: Lab Oncology (IRCH)
Sample Received Time: 18/09/2017 03:47 PM Report Generated Date: 25/09/2017 11:45 AM
Dept / IRCH No: [REDACTED] Recommended By: Dr. Lalit Kumar (HOD)
Lab Reference No: 403

Sample Details : LOI-180917144-CFN (Blood)

CYTO-FNAC/GUIDED SAMPLING

FNAC NO: 403/17 Reg Date: [REDACTED] IRCH NO: [REDACTED]
Site of FNAC: Right pleural thickening Age: 55 years 9 months 10 days
Ref Dept: Medical Oncology Unit Name: [REDACTED]
Lab Name: Oncology Lab Sample Collection Date: 18/09/2017 03:47 PM
Lab Sub Centre: Lab Oncology (IRCH)
Report: Guided aspirate from right pleural thickening shows blood only.

FNAC is inconclusive.

Conclusion: Right pleural thickening : Inconclusive.

Adv/Note: Repeat guided aspirate.

Senior Resident:

(Dr.Pranay Tanwar)
Authorized Signatory



Dr. B. R. AMBEDKAR INSTITUTE ROTARY CANCER

ALL INDIA INSTITUTE OF MEDICAL SCIENCES NEW DELHI

DAYCARE DRUGS ADMINISTERED

12/5/2017

Indoor Reg	IRCH	Ward	ward_name	Consultant Name	Combo6
Patient Name		Age	55	Sex	F
Hb :	gms/	TLC :	/Cum	ANC :	/Cumm
Platelet :					/Cumm
Urea :	mg%	Creatinine	mg%	Bilirubin	mg%
SGOT	I.U	SGPT	I.U	Alk. Ph:	I.U
Diagnosis	CA BREAST	Protocol :	Taxol D1,D8,D15 + Herceptin wkly	CYCLE/DAY	c6
Height:	cms	Weight :	kg	BSA :	1.00

DRUGS AND INFUSION DETAILS

PREMEDICATION GIVEN

Inj Ondansetron 8 mg IVP

Inj Dexamethasone 8 mg

Inj Ranitidine 50 mg

Inj Avil 1 Amp.

CHEMOTHERAPY/IMMUNOTHERAPY GIVEN

Drug Name	Weight	Final Dose	in Unit	Soln.	Infusion	Start/End Time
Inj Taxol		250 mg	500 ml	NS	3 hrs	9:30 ^{am} - 12:30 ^{pm}

SIGNATURE OF
P^hYSICIAN

Combo8

DATE :

SIGNATURE OF
NURSE

SIGNATURE OF
NURSE

Dr. B.R. Ambedkar Institute Rotary Cancer Hospital
ALL INDIA INSTITUTE OF MEDICAL SCIENCES

Height.
Weight
BSA

Name of the patient

Flowsheet Medical Oncology

PATIENT DATE

Date :

Temp Heart Rate Resp Date B.P. Resp. Sys. CVS CNS						
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LABORATORY HB/PCV Platelates (K 1000) WBC (X 1000) ANC ESR Blood Sugar Blood Urea/Creatinine Calcium/Phosphate Uric Acid (mg %) Sodium / Potassium Billirubin (mg %) D/I Total Protein (%) A/G Ratio SGOT/SGPT ALK. Phos (Units) Tumour Markers Others	3/12/16 11.5 522000 8400 5880 26/1.1	9/11 12.7 265000 15000 11500 6/0.9 9.2/3.1 3.5 136/5.1 7.5-4.3 31/57 422	15/11/17 11 666000 4730 2550 15/0.7 9.3/4 4.6 0.5 7.4 38/52 95	20/2 11.1 4.54L 10,200 N63L23 3.9 3.5	16/3 11.0 5.25L 12,500 9346	17/4 11.8 5.02L 13,200 8820
REACTIONS TO THEARAPY Nausea Diarrhea Mucositis Fever Others						
SUPPORT DATA BT / PRP I/O						



Department Of Pathology
All India Institute Of Medical Sciences
Delhi

Tel: +91-11-26588500/26588700; Fax: +91-11-26588500/26588700

Patient Name: [REDACTED]
F/H Name: [REDACTED]
Age/Sex: 55 Y/Female
Clinic/Dept/Unit: N/A/Unit 1
Reg Date: 02-01-2017

Acc. No: 17108
Hosp. Reg. No.: [REDACTED]
UHID No.: ---
Consultant Incharge: Dr. N/A
Reporting Date: 12-01-2017

Histopathology Report

Report Findings:

Excisional biopsy of left axillary lymphnode shows features of a metastatic carcinoma.

Hormonal profile of the tumor is as follows

Estrogen receptor-positive(8/8)

Progesterone receptor-negative(0)

Her 2 neu-negative(1+)

Note: Patient is a known case of carcinoma breast, vide clinical history.

Reporting Incharge: Dr. Vaishali Suri

Reporting SR: Dr. Vikram Raj

Verify By: Dr. Sajida Batool

डा. बी.आर. अम्बेडकर संस्थान रोटरी कैंसर अस्पताल
अ.भा.आ.सं., अन्सारी नगर, नई दिल्ली-110029

सहमति पत्र

के०पंजी०सं०/बहि०रो०वि०सं०	
नाम	Surey Mukhi
पुत्र/पुत्री/पत्नी श्री	M.A. Syam veer Singh
दूरभाष :	Vill. Nagla Meera P.O. Musauli Jalesar Dist- Etah UP

सहमति के लिए सूचना :

चिकित्सा उपचार संवेदाहरण तथा शल्यक क्रिया करने और/अथवा नैदानिक/चिकित्सीय प्रक्रिया के प्राधिकरण के संबंध में।

1. मैं सत्त्वद्वारा, डा.बी.आर. अम्बेडकर संस्थान रोटरी कैंसर अस्पताल, अ.भा.आ.सं. अ.भा.आ.सं. तथा संस्थान के पदाधारी स्टाफ को Surey Mukhi के विषय में निम्नलिखित चिकित्सा उपचार शल्यक क्रिया और/अथवा नैदानिक/चिकित्सीय प्रक्रियाओं के निष्पादन के लिए प्राधिकृत करता हूँ।
2. मुझे आपरेशन/प्रक्रिया आकस्मिक स्थिति उत्पन्न अथवा प्रकट होने पर प्रारंभिक उपचार में समय आवश्यक शल्यक और अन्य आपात प्रक्रिया के अतिरिक्त अथवा उससे अलग जो अपेक्षित समझते हैं, प्राधिकृत करता हूँ।
3. मैं बेहोशी में प्रयुक्त होने वाले निम्नलिखित वर्जित दवाओं (वर्जित दवाएँ दर्शाएँ अथवा नहीं) के अलावा तथा अपेक्षित समझे जाने वाले संवेदनाहारी दवाओं के प्रयोग की भी सहमति देता हूँ।
potent person .. Bx (C) axillary (N) JILA on 2/1/17
4. मैं स्पष्ट करता हूँ कि मैं उच्चरक्तचाप/मधुमेह/रक्त संबंधी गड़बड़ी/हृदय की बीमारी अथवा से पीड़ित हूँ/नहीं हूँ।
5. मैं यह भी स्पष्ट करता हूँ कि मैं किसी भी प्रकार की एलर्जी अथवा दवाई के प्रभाव से पीड़ित नहीं हूँ।
6. मैं पुनः इस प्रकार की औषधियों प्लाजमा और रक्त आधान अथवा किसी अन्य उपचार तथा प्रक्रिया जो आवश्यक समझी जाए के प्रयोग की भी सहमति देता हूँ।
7. आपरेशन के प्रकार और प्रयोजन/अथवा प्रक्रियाओं आपरेशन की अनिवार्यता संभावित वैकल्पिक पद्धतियों उपचार पूर्वानुमान जाँच प्रक्रिया/जाँच के जोखिम तथा जटिलताओं की संभाव्यता तथा मेरी स्थिति/निदान के अनुरूप उपचार के बारे में मुझे पूरी तरह से स्पष्ट कर दिया गया है। उक्त परिस्थितियों में उपचार के बारे में मुझे जानकारी है।
8. मुझे सभी/कोई भी प्रश्न पूछने का अवसर दिया गया है तथा मुझे किसी अन्य चिकित्सक से मिलकर विचार कायम करने का भी विकल्प प्रदान किया गया है।
9. मैं स्वीकार करता हूँ कि किसी भी प्रक्रिया/उपचार से संबंधित परिणाम के लिए मुझे कोई गारंटी अथवा वचन नहीं दिया गया है।
10. मैं आपरेशन अथवा प्रक्रिया के निष्पादन के लिए अपने शरीर के उपयुक्त अंग की चिकित्सा वैज्ञानिक अथवा शैक्षिक उद्देश्य के लिए फोटोग्राफिंग तथा टेलिविजनिंग करने की सहमति प्रदान करता हूँ बशर्ते कि चित्र अथवा उससे संबंधित वर्णनात्मक विषयों द्वारा मेरी पहचान उद्घाटित न हो।
11. प्रगामी आयुर्विज्ञान शिक्षा के उद्देश्य आपरेशन कक्ष में प्रेक्षकों प्रदेशा सहमति देता हूँ।
12. मैं अस्पताल के अधिकारी वर्ग को शल्यक प्रक्रिया/उपचार के दौरान किसी भी ऊतक अथवा हिस्से का, जो अलग किया जा सकता है के निपटाने की सहमति देता हूँ।

गणित करता हूँ कि उपर्युक्त सहमति पत्र में विवरण मैंने पूरी तरह पढ़ लिया है या मुझे मातृभाषा में पढ़कर सुना दिया गया
उपर्युक्त सहमति के लाभ और हानि से भलीभांति परिचित हूँ और पुनः निवेदन है कि उपर्युक्त विवरण तथा मुझे हस्ताक्षर करने
का निशान देने से पूर्ण काट दिया गया था।

रोगी के हस्ताक्षर/

अंगूठे का निशान

नाम

सूरज प्रवी

दिनांक

2/1/17

गवाह के हस्ताक्षर, नाम और पता :

1.

Runa

2.

Msg officer

यदि रोगी नाबालिग और मानसिक तथा शारीरिक विकलांगता के कारण हस्ताक्षर करने में असमर्थ हो।

वास्तविक अभिभावक/अभिभावक के हस्ताक्षर/अंगूठे का निशान
अभिभावक का नाम तथा रोगी के साथ सम्बन्ध

हस्ताक्षर

नाम

गवाह का पता

1.

2.

मैं पुष्टि करता हूँ कि मैंने उपर्युक्त सहमति पत्र में हस्ताक्षरित रोगी को इस आपरेशन/उपचार की प्रकृति तथा प्रभाव के बारे में पूर्ण जानकारी
दे दी है।

प्रभारी चिकित्सक के हस्ताक्षर

नाम

पदनाम

तिथि :



Department Of Pathology
All India Institute Of Medical Sciences
Delhi

Tel: +91-11-26588500/26588700; Fax: +91-11-26588500/26588700

Patient Name: [REDACTED]

F/H Name:

Shyam Veer Singh

Age/Sex:

55 Y/Female

Clinic/Dept/Unit:

Dr. B.R. A. IRCH/N/A

Consultant Incharge: Dr. Sameer Bakhshi

Reg Date:

14-12-2016

Acc. No: [REDACTED]

Hosp. Reg. No.: [REDACTED]

UHID No.: ---

Consultant Incharge: Dr. Sameer Bakhshi

Reporting Date: 19-12-2016

Histopathology Report

Report Findings:

Ultrasound guided biopsy from 'left axillary node' shows a core of reactive lymphoid tissue.
There is no evidence of metastasis in this biopsy.

Reporting Incharge: Dr. Prasenjit Das

Reporting SR: Dr. Kavneet Kaur

Verify By: Dr. Anil Sharma



Department Of Pathology
All India Institute Of Medical Sciences
Delhi

Tel: +91-11-26588500/26588700; Fax: +91-11-26588500/26588700

Patient Name:	[REDACTED]	Acc. No:	[REDACTED]
F/H Name:	Shyam Veer Singh	Hosp. Reg. No.:	[REDACTED]
Age/Sex:	55 Y/Female	UHID No.:	---
Clinic/Dept/Unit:	Dr. B.R. A. IRCH/Unit 1	Consultant Incharge:	Dr. Sameer Bakhshi
Reg Date:	13-12-2016	Reporting Date:	14-12-2016

Cytopathology Report

Report Findings:

Pleural fluid (500ml) shows lymphocytes, reactive mesothelial cells and few clusters of atypical epithelial cells, consistent with a Metastatic Carcinoma.

Note: Patient is a known case of Carcinoma Breast - vide clinical history.

Cancer Category:

- ☐ **UNS:** Specimen Inadequate For Opinion
- ☐ **NEG:** No Evidence Of Cancer In This specimen
- ☐ **INC:** Further Evidence As Indicated Is needed To rule Cancer In or Out
- ☒ **POS:** **Diagnostic For Cancer Correlation with Clinical, Radiological and other Investigation is mandatory**

Reporting Incharge: Dr. Asit Ranjan Mridha

Reporting SR: Dr. Tripti Nakra
Verify By: Dr. Sudha

12/14/2016 6:00 PM

PT. NAME

55 YEARS / F

REF. BY

DR. ANIL SARASWAT (MD)

30TH NOVEMBER, 2016

ULTRASOUND WHOLE ABDOMEN

Liver is normal in size, outline with normal echotexture. No obvious focal parenchymal lesion is seen. Hepatic veins and portal venous radicals are normal. IHBR is not dilated.

Gall Bladder is not visualized -- History of cholecystectomy.

CBD is not dilated. Portal vein is normal.

Pancreas is normal in outline and showing normal echopattern. No evidence of any focal lesion or calcification is seen. MPD not dilated.

Few lymph nodes noted in relation to body of pancreas and head of pancreas measuring 15mm and 13mm respectively.

Spleen is normal in size and shows normal echopattern. No focal lesion is noted.

Both kidneys are normal in outline, shape, location, size and reveal normal echogenicity. The CM differentiation is maintained with maintained cortical thickness. No calculus or hydronephrosis is noted on either side. B/L ureters are not dilated.

No ascites is noted in the peritoneal cavity.

No evidence of any pleural effusion was evident at the costophrenic angles.

Urinary Bladder is well distended with normal outline. No focal wall pathology of filling defect seen. No definite calculus is noted.

Uterus is normal in size and echopattern. No myometrial / endometrial pathology is noted. No focal fibroid is noted. Bilateral ovaries appear normal. No adnexal pathology or free fluid is seen in POD.

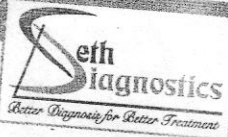
IMPRESSION:

- FEW LYMPH NODES NOTED IN RELATION TO BODY OF PANCREAS AND HEAD OF PANCREAS MEASURING 15MM AND 13MM RESPECTIVELY.

Please correlate clinically.

DR. KANIKA GOEL
MD (RADIODIAGNOSIS)


DR. NITEN SETH
MD (RADIODIAGNOSIS)



Raj Hari Memorial
SETH DIAGNOSTICS

1/188-A, B. M. SARKAR ROAD, DELHI GATE, AGRA-282 002

PH.: 0562-2523333 / 98370 26607

PT. NAME

[REDACTED]

REF. BY

DR. ANIL SARASWAT (MD)

55 YEARS / F

30TH NOVEMBER, 2016

ULTRASOUND CHEST & BREAST

ULTRASOUND CHEST

Gross right pleural effusion is noted. Right pleural effusion is causing compression collapse of underlying lung parenchyma.

Multiple pleural deposits are noted measuring 15-20mm.

Left pleura are clear. No obvious pleural fluid / thickening is noted.

No subdiaphragmatic collection is noted.

ULTRASOUND BREAST

Multiple nodular deposits are noted in the presternal region measuring 25 x 12mm and 25 x 15mm --- Metastasis.

Right breast is not visualized -- History of surgery.

Left breasts reveal normal glandular and fatty tissue. No obvious intramammary calcification is noted. No definite focal lesion is noted.

Subareolar regions appear normal.

Please Correlate Clinically.

DR. KANIKA GOEL
MD (RADIODIAGNOSIS)


DR. NITEN SETH
MD (RADIODIAGNOSIS)

Lab Serial No.

Patient Name

Category

Referred By

[REDACTED]
[REDACTED]
DELHI
Dr. A K ARYA

Age/Sex

Reg. Date

Report Date

Accession No.

Test Name

: 55 Yrs /F

: 02-Dec-2016 18:54:02

: 03-Dec-2016 03:59PM

: 18 F FDG PET/CT
WHOLE BODY

DEPARTMENT OF PET CT

F18-FDG WHOLE BODY POSITRON EMISSION TOMOGRAPHY WITH CONTRAST ENHANCED CT SCAN

F18 - FDG Positron Emission Scan with a diagnostic high resolution CT scan was performed from the vertex to mid-thigh using the GE Discovery STE dedicated PET scanner with 32 slice/sec Multidetector Computerised Tomography (MDCT). Oral contrast was administered for bowel opacification. Non-ionic intravenous contrast injection was administered. Specific dose optimization protocols were used on CT to reduce radiation dose to the patient. A separate sequence with breath hold was performed for lung. A semiquantitative analysis of FDG uptake was performed by calculating SUV value corrected for dose administered and patient body weight. The blood sugar at the time of tracer injection was 123mg/dl.

Patient is a known case of carcinoma right breast, post MRM (2004), post chemotherapy and post radiotherapy status. PET CT scan is being done for post treatment surveillance.

The overall biodistribution of FDG is within normal physiological limits.

Head & Neck:

Brain:

The infra tentorial and supratentorial brain parenchyma appears normal and show normal physiological FDG uptake. No focal lesion or abnormal focal uptake is noted.

It may kindly be noted that all brain metastases may not be apparent on a PET CT scan and an MRI head may be performed where clinically indicated.

Sinuses: Well pneumatized.

Neck:

Normal physiologic uptake noted in the nasopharynx, oropharynx, hypopharynx and larynx. Parotid and submandibular glands demonstrate normal metabolic activity.

The thyroid gland is sharply demarcated and shows homogenous pattern on CT scan. No abnormal FDG uptake is seen in the thyroid and the neck structures.

In case of any discrepancy due to typing error, kindly get it rectified immediately. This is professional opinion, not a diagnosis.

H.O.D

HOUSE of DIAGNOSTICS

Lab Serial No. : [REDACTED]

Patient Name : [REDACTED]

Category : DELHI

Referred By : Dr. A K ARYA

Age/Sex

Reg. Date

Report Date

Accession No.

Test Name

: 55 Yrs /F

: 02-Dec-2016 18:54:02

: 03-Dec-2016 03:59PM

: 18 F FDG PET/CT
WHOLE BODY

No evidence of cervical lymphadenopathy noted.

Thorax:

Right breast is not visualized – post MRM status.

FDG avid (SUV max –16.8) heterogeneously enhancing soft tissue density area is noted around sternum extending posteriorly and involving adjacent mediastinum and closely abutting adjoining vessels (predominantly arch of aorta, superior vena cava and innominate vein) and pericardium with loss of intervening fat planes at few places. FDG avid (SUV max –6.3) sclerotic changes are noted in the adjoining part of sternum with cortical irregularities.

Small non enhancing hypodense area is noted in the confluence of superior vena cava – likely suggestive of thrombus.

The heart and mediastinal vascular structures are well opacified with I/V contrast.

FDG avid (SUV max –11.7) lymph nodes are seen in left intra-clavicular, left axillary, anterior diaphragmatic, prevascular, right paratracheal, AP window, subcarinal & bilateral hilar regions and around thoracic aorta, largest measuring ~ 28 x 23 mm in right hilar region.

Significant right sided pleural effusion is noted with collapse consolidation of underlying segments of lung showing patchy FDG avidity.

FDG avid (SUV max –11.1) heterogeneously enhancing multiple focal pleural thickening / deposits are noted in both lungs (Right>>Left).

Abdomen:

The liver is normal in size and attenuation pattern. No focal intra hepatic lesion seen. The intra hepatic biliary radicals are not dilated. The portal vein is normal. No abnormal FDG accumulation seen in the liver parenchyma.

The gall bladder is not visualized (post cholecystectomy status).

The spleen is normal in size and demonstrates physiological FDG uptake.

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Lab Serial No. : [REDACTED]
Patient Name : [REDACTED]
Category : DELHI
Referred By : Dr. A K ARYA

Age/Sex : 55 Yrs /F
Reg. Date : 02-Dec-2016 18:54:02
Report Date : 03-Dec-2016 03:59PM
Accession No. :
Test Name : 18 F FDG PET/CT
WHOLE BODY

FDG avid (SUV max -7.3) minimally enhancing hypodense areas are noted in the head and periampullary regions of pancreas, largest measuring ~ 19 x 18 mm in periampullary region.

Both adrenal glands demonstrate near normal size, uniform homogenous enhancement on CT and no abnormal FDG uptake.

Bilateral kidneys appear normal in size, shape and attenuation and FDG uptake. No evidence of hydronephrosis is noted.

The stomach, small bowel and large bowel loops appear normal in caliber and fold pattern.

FDG avid (SUV max -10.2) lymph nodes are seen in left external iliac region measuring ~ 28n x 19 mm.

There is no evidence of ascites.

Urinary bladder is partially distended.

Prostate appear unremarkable.

Non-FDG avid bilateral inguinal lymph nodes are seen with preserved central fatty hilum (reactive).

Musculoskeletal system:

FDG avid (SUV max -9.9) focal ill defined lytic area is noted in right parietal along inner table.

FDG avid (SUV max -5.3) lytic sclerotic areas are noted in the inter trochanteric region of left femur, pelvic bones, multiple dorsolumbar vertebrae, left sacral ala & few ribs.

Degenerative changes are seen in the spine. Rest of the bones under survey otherwise appear normal.

OPINION: In a known case of carcinoma right breast PET CT scan findings are suggestive of:

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Lab Serial No. : [REDACTED] Age/Sex : 55 Yrs /F
Patient Name : [REDACTED] Reg. Date : 02-Dec-2016 18:54:02
Category : DELHI Report Date : 03-Dec-2016 03:59PM
Referred By : Dr. A K ARYA Accession No. :
Test Name : 18 F FDG PET/CT
WHOLE BODY

- FDG avid heterogeneously enhancing soft tissue density area around sternum extending posteriorly and involving mediastinum with involvement and extent as described - likely metastatic.
- FDG avid left intra-clavicular, left axillary, anterior diaphragmatic, mediastinal, bilateral hilar and left external iliac lymphadenopathy - likely metastatic.
- Significant right sided pleural effusion with collapse consolidation of underlying segments of lung showing patchy FDG avidity.
- FDG avid heterogeneously enhancing multiple focal pleural thickening / deposits in both lungs (Right>>Left) - likely metastatic.
- FDG avid minimally enhancing hypodense areas in the head and periampullary regions of pancreas - likely metastatic.
- FDG avid focal ill defined lytic area in right parietal along inner table - likely metastatic.
- FDG avid skeleton lesion as described - likely metastatic.

Associated findings: Small non enhancing hypodense area in the confluence of superior vena cava – likely suggestive of thrombus.

Please correlate clinically and histopathologically.

End of Report

In case of any discrepancy due to typing error, kindly get it rectified immediately. This is professional opinion, not a diagnosis.

H.O.D

HOUSE of DIAGNOSTICS

Lab Serial No. : [REDACTED]
Patient Name : [REDACTED]
Category : DELHI
Referred By : Dr. A K ARYA

Age/Sex : 55 Yrs /F
Reg. Date : 02-Dec-2016 18:54:02
Report Date : 03-Dec-2016 03:59PM
Accession No. :
Test Name : 18 F FDG PET/CT
WHOLE BODY

Pathak

Dr. Vivek Pathak
Senior Consultant (Nuclear Medicine)
M.B.B.S., D.N.B., M.N.A.M.S.
DMC Reg. No. 45786

Abhi

Dr. Abhijeet Tiwari
Consultant Radiologist
M.B.B.S., D.N.B. (Radio-Diagnosis)
DMC Reg. No. 51466

Register to MOC
↓ Dr. Anil Bhatia
Dr. Sameer Bhatia
on 8/12/16
Anil
RN-5.

In case of any discrepancy due to typing error, kindly get it rectified immediately. This is professional opinion, not a diagnosis.

Lab Serial No. [REDACTED]

Patient Name [REDACTED]

Age/Sex : 55 YRS / F

Referred By : Dr. SELF

Test Name : UREA

Center :

Category : DELHI

Reg. Date : 03-Dec-16 09:34 AM

Sample coll. Date : 03-Dec-16 10:37 AM

Report Date : 03-Dec-2016 11:33 AM

Test Name	Observed Value	Unit	Biological Ref Interval
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BIOCHEMISTRY

SERUM CREATININE , Serum by enzymatic	1.10	mg/dl	0.6-1.20
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Laboratory is NABL Accredited for serum Creatinine.

Remarks: Please correlate clinically

BLOOD UREA , Serum by Urease	26.22	mg/dl	13-40
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Laboratory is NABL Accredited for serum test for Urea.

Remarks: Please correlate clinically

*** End Of Report ***

Dr. Geetanjali
Consultant Pathologist
M.B.B.S., M.D. (Pathology)
DMC Reg. No. 52507

Lab Serial No. [REDACTED]

Patient Name [REDACTED]

Age/Sex : 55 YRS / F

Referred By : Dr. A K ARYA

Test Name : CBC

Center :

Category : DELHI

Reg. Date : 03-Dec-16 09:43 AM

Sample coll. Date : 03-Dec-16 09:46 AM

Report Date : 03-Dec-2016 11:33 AM

Test Name	Observed Value	Unit	Biological Ref Interval
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HAEMATOLOGY

CBC

TOTAL LEUCOCYTE COUNT	8.40	X 10 ³ / μ L	4.0-10.0
DIFFERENTIAL LEUCOCYTE COUNT			-
NEUTROPHILS	70	%	40-80
LYMPHOCYTES	23	%	20-40
MONOCYTES	06	%	2-10
EOSINOPHILS	01	%	1-6
BASOPHILS	00	%	0-1
ABSOLUTE NEUTROPHIL COUNT	5.88	X 10 ³ / μ L	2.0-7.5
ABSOLUTE LYMPHOCYTE COUNT	1.93	X 10 ³ / μ L	1.0-4.0
ABSOLUTE MONOCYTE COUNT	0.5	X 10 ³ / μ L	0.2-1.0
ABSOLUTE EOSINOPHIL COUNT	0.08	X 10 ³ / μ L	0.04-0.44
ABSOLUTE BASOPHIL COUNT	0.0	X 10 ³ / μ L	0.00-0.30
TOTAL RBC	4.75	million/ μ L	3.8-4.8
HEMOGLOBIN	11.5	gm/dl	12.0-15.0
PLATELET COUNT	522	X 1000 / micL	150-450
HEMATOCRIT	38.0	%	36-46
MEAN CORPUSCULAR VOLUME (MCV)	80.0	fL	83-101
MEAN CORP. HEMOGLOBIN (MCH)	24.2	pg	27-32
MCH CONCENTRATION (MCHC)	30.3	g/dl	31.5-34.5
RED CELL DIST. WIDTH (RDW-CV)	16.4	%	11.5-13.1
RED CELL DIST. WIDTH (RDW-SD)	48.5	fL	39-46

House Of Diagnostics laboratory is NABL Accredited for Complete Blood Count (CBC)

Remarks: Please correlate with clinical conditions.

Sample Type: EDTA Whole Blood Sample

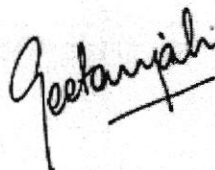
Technology: Fully Automated Haematology Analyzer: SYSMEX XN-550

Methods: Total Leucocyte Count: Flow Cytometry; Differential Leucocyte Counts: Fluorescence Flow

Cytometry/Microscopy; Hemoglobin: Cyanide Free SLS Method; Platelet Count, Total RBC: Impedance Method with Hydrodynamic Focussing;

Haematocrit, MCV, MCH, MCHC: Calculated

*** End Of Report ***



Dr. Geetanjali
Consultant Pathologist
M.B.B.S., M.D. (Pathology)
DMC Reg. No. 52507

डॉ० बी० आर० अम्बेडकर संस्थान रोटरी कैंसर अस्पताल, अ.भा.अ.सं., नई दिल्ली
Dr. B.R.A. Institute Rotary Cancer Hospital, AIIMS, New Delhi
MEDICAL ONCOLOGY

Date: 8.12.2016

[redacted] 55/female

w/o: Shyam Veen Singh

[redacted]

Menopause > 12 yrs

Breast

2004 - diagnosed with adenocarcinoma (R) breast (T3N0M0) in 2004
and treated at SN Medical College and Hospital Agra

Presented with painless lump in (R) breast for 3 years
which was ~ 6cm size and located in retroareolar area.



FNA: s/o papillary adenocarcinoma breast



ER, PR, Her 2 not done

Received 4 # chemotherapy IMF at SNMC-Agra (↓ RT dept)

on 8.6.2004

Sfor

29.6.2004

Metho

29.7.2004

5Fu

10.8.2004



MRM on 2.9.2004 at SNMC Agra

EBRT 50 Gy / 25 # / 4wks (25.8.2004 to 3.12.2004)

Tamoxifen 5yr (till 2010)



on 6 monthly follow up



Pain (R) upper abdomen
Retrosternal chest pain
Pain (R) upper limb
Dyspnea on exertion

1½ - 2 months

USG - abdomen (30.11.2016): fav LN in body of pancreas and head of pancreas (1.5cm and 1.3cm)

USG - chest/breast (30.11.2016): Gross (R) pleural effusion and collapse of underlying lung parenchyma
X^{lt} pleural deposits (1.5 to 2cm)
X^{lt} nodular deposits in the presternal region
2.5 x 1.2 cm and 2.5 x 1.5 cm²

PET-CT (2.12.2016) → FDG avid mass around sternum (SUV 16.8) extending posteriorly and involving adjacent mediastinum/abuts adjoining vs. and pericardium = loss of intervening fat planes. FDG avid sclerotic changes are noted in the adjoining part of sternum with cortical irregularities (SUV max 6.3)

~ small non-enhancing hypodense area is noted in confluence of SVC ? thrombus

~ FDG avid LN in (L) intraclavicular, (L) axillary, ant-h-diaphragmatic, prevascular (R) paratracheal, AP window, subcarinal and BL hilar regions (2.8 x 2.3cm)

~ (R) pleural effusion = collapse consolidation of underlying segments

~ FDG avid X^{lt} focal pleural thickening or deposits in both lungs (R) > (L)

~ FDG avid hypodense areas noted in head and perianastomosis pancreas (1.9 x 1.8cm)

(L) external iliac region FDG avid nodule (2.8 x 1.9cm)

~ FDG avid focal ill defined lytic area (R) parietal along inner table (SUV max 9.9) and lytic sclerotic areas are noted in the intertrochanteric region (L) femur, pelvic bones, X⁶ dorsolumbar vertebral bone, (L) sacral ala and ribs

Past history: No h/o HTN, DM, TB, asthma
h/o cholecystectomy in 2010

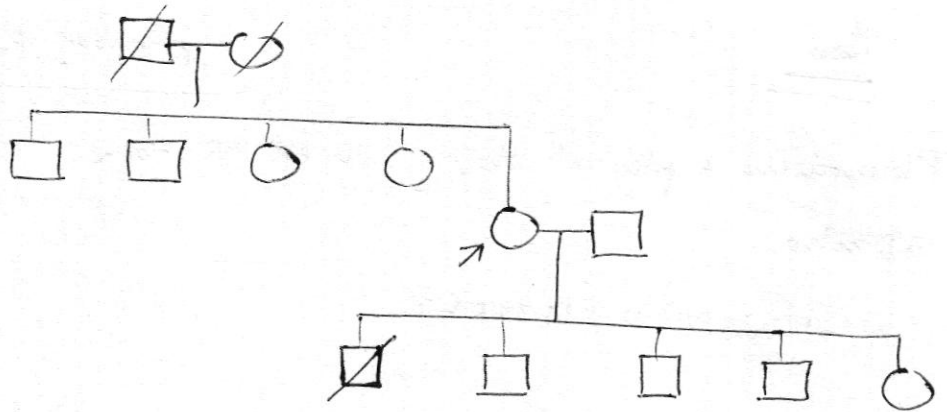
Personal history: Menopause > 12 yr back (prior to treatment for breast cancer)

Menarche: 12 yr age

Married: 18 yr age

First child: 20 yr age

Family history:



no h/o breast or ovarian ca in family

Socio-economic history:

Husband: Teacher
(retd)

lives in joint family & son
Elder son: Farmer

No insurance support

Ht: 150cm
Wt: 61.3kg
BSA:

L/E: (R) breast - MRM scan

(L) breast - (W) NAC
no lump
axilla (u)

Pallor ⊖

Facial puffiness ⊖

RS: BS ↓ (R); localized crackles - infrascapular

PA: soft R ⊕ L 500 LCo

CNS: No FND

CVS: S2 ⊕

Plan:

Δ: Metastatic Ca (R) breast
probably ER ⊕ Her2 ⊖

X Pleurodesis + pleural biopsy for ER, PR, Her2

• AD echo

• Ca²⁺, PTH, VitD / LFT/RFT

Biopsy → Room (A9) any feasible site / (L) axillary LN
ER, PR, Her2 rec on biopsy

• Start Anastrozole 1mg PO OD
Zoledronate Amg Q 3/12
Calcium / vitamin D

• Pleural fluid tap — Room (15)
+ cytopath

WAB
follow up on 29/12

P

29/12/16

55/F.

Q ca breast - 2004 ✓
T3N0M0

ER
PR
HER2/neu } ?!

↓
4th IMF ✓

↓
MRM - 2/1/2004

↓
EBRT 5044/25H/4WKS

↓
Tamoxifen - 5 years (un 20102)

ADH - Prostate with

- Pain @ Abdomino
- Perosteal chest pain
- Pain @ upper limb
- Dyspnea on exertion

PET-CT - soft tissue density and
around sternum extending posteriorly and
involving mediastinum with involvement of

- infracardiac, left axillary, and diaphragmatic
mediastinum, B/L hilar. p left 6th rib
LVE

- R+ sided pleural effusion &
collapse consolidation of underlying
segment.

- Focal pleural thickening in both
lungs

- Bony mets.

metastatic ca breast.

visceral embolic pulmonary
effusion

wt - 60.70 kg.

HT - 150cm

BIA - 1.5ml

{ - cough.
- svc. thrombus }

Plan

= SA Paclitaxel - (90mg. weekly)

- Anti coagulation for svc

- Bx from ycthesia soft tissue (CT guided)

- INI. zoled. 3 monthly

SIOA Anastrozole.

Plan

5/1/16

metastatic ca Breast & visceral cancer

Heumari Gms. PE

Bare male &

SA - Paclitaxel (Reinforced 1# on 29-12-16)

CBC

12.5/7390/6,40,000

(4-1-17)

Plan

2nd weekly dose

PACT

on 12/1/17

16/1/17

16-01-2017

Excision Pbx (L) AX-LN → met. carcinoma
17108 ER 5/8 PR 0/8 Her2 neu: 1(+)

post 2# Pachi

Cough and orthopnea persists : CXR urgent

(R) BS ↓

? SVC thrombus on PET film

Q/W AB

Pachi = 120mg → Q3 weekly from next cycle onwards

Room (16) for pleural tap

FN → 23.01.2017

Discuss PET for ? SVC thrombus
DIRC

DIRC:-

Stenosis at right brachiocephalic vein + SVC approx 2cm
radiologically specification s/o thrombus.

→ may be a site of prior central line.

→ radiologically significant

Clinically may not be interpreted due to collaterals
development.

AE

30/1/17.

MBC

SR+ (8/8)

PR-

HER2 neu -

Prevent effusion

SA taxam

Post 3 a weekly

Adm

Start 3 weekly Pacli

1 250 mg

for - 20/2/17

Q

20/2/17.

Cycle 3 Pacli

~~W~~

1st 3

symptoms

but

Cy Pacli

~~W~~

Cy

- 17/4.

~~W~~

ES - 11/5.

Completed C6 Paclitaxel -

CAR 8/0 (8) pleural effusion

Adv.

BR patient

- Palbociclib

+

- letrazole



~~2/7~~

2/8

Post 2 cycles
symptoms - ok

Palbociclib/letrazole

Adv.

18/9/12

18/9

Post 3 cycles

Palbo/letrazole

symptoms - congs.

Adv.

cy. ✓

fm.

16/10/17

CSCC chest/Abdomen

fm

16/8/17

MBC

P/ S-A Taxol $\rightarrow$! SO/PA

on Pulbo c66b + letrozole

So for PA #

Tolerating well

NO FN

Clinical no SOB

no new symptoms

Imaging - Await

FNAC from

Rt pLent biopsy $\rightarrow$ Blood only

Act

Continue Pulbo + Letrozole

T14 P.O

NIA 1st

T2

DIRC
12/12/17

current scan

- ~~sol. can~~

- Bilateral Pl. effus. -

- No parenchymal dis. only

- (L) adrenal bulky but unlikely mets

- Pleural thickening @ R. is features of

empyema & lobar consolid.

(Wakeup for TB)

[Pleural effusion has mildly ↓ ed]

- Consolid. has ↓ ed

- The para-aortic & node ~~has~~ is subcon. nodular

overall - Stable disease

18/12/2017.

Q

Plan

- Bilateral pigtail drainage.

↓

- Send for cyt. path

- Wakeup for TB.

cont Palbociclib + letrozole.

↓

Re

Next cycle

→ Anthracyclines (EC) @ 75%
don

by 75RMO

21/12

PD

Hormone - +ve

Response - smokes

Symptom burden.

< hyl.

Adh

shut EC

fu

11/1

Symptoms better.

Toleram - on

Adh.

C₂ EC

fu

22/2/18

Δ: MBC (R) breast - post MRM (2004)

ER 8/8

PR 0/8

Hu2: 1 (+)

~~CINV gr~~

post Pacli 4 #

Palbo + Letrozole

EC 3 # given

symptomatically better

Adh:

plan EC # 4

fu on 19/3/2018

response
CT due on 20/3

09/04/18

✓ Post 4 # BC - disease. shut - so

Now P/BC #5 (19/03/18)

9.

SOD increased in last 3 days

on excretion

MYA-III

ELA - (09/04/18)

- pass' R name program
- Sun. tech

CXR PA, 09/04/18

① moderate effusion

(no previous CXR available
for comparison)

Clinical

2 PD

12/04/18

P/F - not-tappable

Clinical

- SOB on exertion / easy fatigue
other vitals OK (no postural variation)

- CBC-OK

CT-d

ADW

① CECT M/4A/18

② P/F top +
cytology

③ F/U 12/05/18

④ W/14 C#6 EC

SM
SM

ADW

① CT-date

② To go for C#6

③ F/U. ur

IRCH No. 194632

Reg. Date-08/12/2016

Clinic Adult Medical Oncology Clinic

Clinic No. 23195/2016

Deptt. MEDICAL ONCOLOGY

General



Name SURAJ MUKHI

UHID-102423010

W/O- MR. SHYAM VEER SINGH

Sex/Age F/55Y

Phone No. 9411050728

Room 5 (Shift Morning)

Address VILL- NAGLA MEERA PO- MISAULI, JALESAR DISTT- ETAH, UTTAR PRADESH, INDIA

RT-II-101009

Suraj mukhi
55/F28/5/18

55/F, K1C10 @ met. Breast ca

Came with c/o

- headache

- vomiting

- Restlessness

x 7 days

Pos H/O

No H/O HTN, DM, TB, Asthma, etc.

H/O ca Breast - dxed in 2004.

Personal H/O - No alcohol

Menstrual H/O - post menopausal (1248 bad)

Family H/O - no malignancy in family member

Treatment & Investigation H/Odxed as $\textcircled{R}$ Breast T3 Nomo. 2004
ERPos MACT $\rightarrow$ MRM $\rightarrow$ EBRT $\rightarrow$ HT
10/8/2004 2/9/04 w.ef. 25/8/04-3/12/Sys H/O
20/10PET-CT - 2/12/16

- Mediastinal mass
- SVC ? thrombus
- Multiple LN pathy - ($\textcircled{R}$ infraclav, axilla, diaphragmatic $\textcircled{R}$ para)

- $\textcircled{R}$ PE

- Bony met

60 paclitaxel - (wkly)SD $\downarrow$ changed to

Palbociclib letrozole - Dec/2017

SD $\downarrow$ changed toEC - 60 - April/2018 $\downarrow$ CECT - Brain - 27/5/18

- multi-focal intra-axial lesion & perilesional edema in bil. cerebral & cerebellar regions
- S/O - met